The lecture in two minutes.

Intro to the Sales Process and Prospecting | Brianna Salinas | 500 Eurasia Batch 11 Full recording: 2h 10m 50s. Includes an opening cohort discussion before the main lecture.

The central idea

A stronger product does not automatically produce a sale. Choose a reachable customer with an urgent problem, understand how they buy, and help the right people get to a decision.

1. A pilot is a beginning, not proof of traction.

The opening discussion stresses conversion from pilots to paying customers. Agree what success means, stay close during the test, and learn why it does or does not convert. Fewer supported pilots can be more useful than many abandoned ones. **Replay: 00:18:03-00:19:36.**

2. Do not sell to everybody.

Start with a narrow market you can reach and serve well. Define the ideal customer using the problem, existing tools, buying process and reason to act - not just industry and size. **Replay: around 01:17-01:40.**

3. Your pipeline is not their buying process.

A demo may be finished while the customer still has to build an internal case, align colleagues and obtain approval. Discover those steps instead of assuming interest means readiness. **Replay: around 00:37-00:56.**

4. A contact is not a qualified opportunity.

Look for customer fit, a likely pain, a trigger, the right buyer and a credible reason to speak now. Prioritize serious need and a realistic route in. **Replay: around 01:41-01:55.**

5. Map people, not only job titles.

Know who approves spending, who tests feasibility, who influences users and who can explain the organization from inside. One friendly person may not control the decision. **Replay: around 01:56-02:10.**

Four traps to avoid (00:30:59-00:33:39): targeting everyone; hiring salespeople before you have learned how to sell; building before confirming demand; and competing only on price. Strong onboarding can itself be a differentiator.

For MediMind: lead with the business problem and demonstrable value, not AI features or a lower price alone (00:29:39; 00:32:32). Use the free parallel system to prove an agreed improvement to a qualified hospital. This application is ours, not a lecturer-endorsed promise.

Replay only what you need.

Navigation uses the original video clock. Ranges are approximate topic locations, not exact slide-change boundaries.

Where	What is there	Why it matters
00:02-00:11	Opening: assignments, metrics and distribution	Finish the foundations so the experiment period is spent executing, not still deciding who to sell to.
00:11-00:20	What changed after week one; pilot conversion	Team alignment, written flows, agreed success plans, and supported pilots that become paid customers.
Around 00:25	Main lecture begins	Title, speaker introduction and agenda.
00:29-00:36	B2B sales and common mistakes	Multiple stakeholders, complexity, founder learning, and the danger of treating everybody as a customer.
00:37-00:54	The buyer's journey	Learning, evaluating, internal approval, friction and loops.
00:54-01:12	Map the customer's real process	Questions about people, demos, pilots, alternatives, approvers and the contract signer.
01:14-01:22	Adoption and beachhead markets	Find early customers who need the change, can be reached and can make a decision.
01:22-01:41	Ideal customer profiles and buying signals	Move beyond broad categories. Includes a multi-location healthcare example around 01:34.
01:40-01:44	Pain versus ease of reach	Rank 5-7 possible customer profiles; match your message to the recipient's responsibilities.
01:44-01:49	Break - skip for studying	Room conversation, not the lesson. Teaching resumes around 01:49.
01:49-01:55	Contacts, lead sources and qualification	Build a researched shortlist rather than treating a database of names as a pipeline.
01:56-02:10	Stakeholders and buying/selling alignment	Economic buyer, technical buyer, influencer and coach; map relationships and the next decision.

If you have only a few minutes

Read page 1. Then revisit **00:18:03-00:19:36** for the pilot warning, the **01:34** healthcare example, and the stakeholder section from roughly **01:57**. Use the transcript to find an exact phrase before replaying.

Source boundary: pain points and detailed value-proposition work were deferred until after lunch (02:10:13), outside this recording. The guide summarizes this video and its visible slides; it does not independently validate market claims or current tool availability.

Help the hospital buy.

Lecture framework: the buyer's journey is not the same as the seller's activity list. Sources: roughly 00:37-01:12; buying-process slides 10-17.

The lecture's buying sequence is: **learn → understand needs → explore options → evaluate options → make the internal case → finance/purchasing → confirm with the boss → agree payment terms.** Actual deals can loop back when people, budget or requirements change.

Customer's question	Useful MediMind response - our application
Do we have a problem worth fixing?	Find one recurring breakdown, who experiences it, and what it costs in time, rework or missed revenue. Establish a baseline; do not invent savings.
What would a better result look like?	Agree the workflow to test and the evidence that would demonstrate an improvement.
Can this work in our hospital?	Deliver the agreed parallel evaluation with named staff, test instructions, a support owner and a review date.
Can I convince the other people involved?	Give the internal sponsor a short results summary, known gaps, a clear business case and a safe rollout outline.
Can we approve and purchase this?	Identify authority, procurement, technical/security checks, commercial terms and the remaining decision steps before assuming the deal is won.

Ask about a real past purchase.

Who first noticed the problem? How many demos were needed? Which alternatives were considered - including building internally? Was a proposal or pilot required? Who joined at each step? Who approved it? Who signed? What nearly stopped it? The lecture recommends understanding both the **person** and the **process**.

Demo rule, 00:57:15-00:59:02: discover the pain and buying process first; then show how the relevant problem gets solved and agree next steps. The lecturer generally favors a prepared, separate demo, while allowing exceptions for experienced sellers and repeatable transactional sales.

Progress needs buyer-side evidence

Activity: we sent a demo.
Progress: the hospital named an evaluator, agreed success criteria and booked the next step.
This distinction is our practical interpretation of the buying-versus-selling framework.

Choose a market you can win first.

Lecture framework: beachhead market + ideal customer profile (ICP). A beachhead is a focused first market, not your ultimate market size. Sources: roughly 01:17-01:41.

The beachhead checklist

The slide lists nine tests: customers buy similar products; they communicate with one another; budget already exists; the need is urgent; competition is limited; access is direct; decisions are relatively fast; customers will try something new; and success can be measured. The slide's **three-month maximum sales cycle** is a selection heuristic, not a proven hospital buying timeline.

What to define	MediMind working hypothesis - validate it
Specific segment	Reachable private hospitals or hospital groups in Georgia, outside the anchor partner, with an identifiable buying organization.
Pain and use case	One repeated cross-department problem that leadership and staff both recognize; avoid a generic claim that every hospital needs everything.
Current environment	Existing software, vendors, renewal constraints and integration gaps. Confirm these in discovery.
Buyer and process	Budget authority, internal sponsor, clinical/operations evaluators and technical approvers. Record who decides and how.
Evidence of timing	A relevant renewal, expansion, leadership change or an active operational initiative. These are research hypotheses, not known facts about a target.
Readiness to test	Named evaluators, an achievable scope, required approvals and a willingness to agree how results will be reviewed.

Use custom data to find a reason to act.

The healthcare example around **01:34** uses locations, expansion, acquisitions, hiring and current software as signals. The example is about US multi-location healthcare businesses; do not copy its geography, size bands or buyer titles into MediMind without validation.

Prioritize pain + access

Rank **5-7 possible customer profiles** by pain and ease of reach (01:40:12). Start with serious need and a realistic route in. Good targeting still needs useful messaging: offer relevant insight or content, and speak to the recipient's actual responsibility - not just their job title (around 01:30-01:34; 01:42).

Know the people behind the decision.

Lecture framework: stakeholder roles and relationships. Source: roughly 01:56-02:10; stakeholder slides 48-53.

Role	What to learn	Possible hospital counterpart
Economic buyer	Who can approve spending and is accountable for the business result?	Owner, CEO or a delegated executive. Confirm authority; do not infer it from title.
Technical buyer	Who assesses feasibility, integration, security and implementation constraints?	IT/security lead and other designated reviewers.
Influencer	Whose opinion changes whether colleagues support or resist the purchase?	Clinical, nursing, operations or finance leaders and respected users.
Coach	Who can explain priorities, decision rules and internal relationships?	A trusted hospital insider willing to help us understand the organization.

A warm introduction opens a door; it does not qualify the deal.

Our distinction for MediMind: a **connector** introduces us; a **coach** explains the organization; a **champion** actively helps move the evaluation forward; the **buyer** approves spending. One person can play several roles, but we need evidence of each.

Lead with value without ignoring technical risk.

The lecture warns that premature technical conversations can distract from the economic case. For a hospital evaluation, that is **not** a reason to bypass necessary clinical, IT, security or data-access approvals. Establish the business problem and bring the right reviewers into the agreed evaluation process.

One account, one simple relationship map

Record: **role → priority → influence → relationships → next action.** Bring key approvers in early (around 02:08). Include end users: an executive can buy a tool that staff refuse to use (around 02:07). Ask your coach who needs to agree and what each person needs to see.

Messaging implication - our application: the CEO needs a business case, staff need a better daily workflow, and IT needs a feasible implementation. A fast innovation pilot does not prove access to the long-term budget; identify who funds the eventual purchase (lecture: around 01:20).

Turn the lesson into working outputs.

Separate lecture prompts from suggested actions. Do not confuse completing a document with proving customer demand.

Prompts actually visible or spoken in this recording

Opening, 00:11:33: name one significant change made after week one.
Session outputs, 00:24:50-00:25:49: ideal customer profiles and customer value propositions; talk about value rather than features. These feed later outreach, discovery and demo sessions.
Buying process, around 00:55-01:12: answer the person/process questions and compare the customer's buying steps with your selling steps.
ICP, around 01:37-01:40: identify useful custom data points.
Prioritization, 01:40:12: list 5-7 potential customer profiles and rank them by pain level and ease of reach.

The opening asks teams to finish foundations and settle on metrics during that program week - not a new deadline today. Detailed pain-point/value-proposition work is announced for after lunch and is **not included in this recording**.

Suggested MediMind output	What a useful first draft contains
1. ICP + customer value promise	One segment, its pain, the buyer, timing and exclusions. State the valuable change for that customer, rather than a feature list.
2. A buying-process map	Start with the anchor partner's real experience, then test it with a new hospital. Note each role, decision, approval and delay.
3. A researched shortlist	For each candidate: fit evidence, likely problem, trigger, buyer, route in and next action. Unknown fields stay unknown.
4. A stakeholder map	Economic buyer, technical reviewers, influencers and a coach. Identify who can organize a real test.
5. A one-page evaluation plan	Agreed scope, baseline, success criteria, named testers, responsibilities, approvals, support and a dated results review.
6. A decision record	What improved, what did not, remaining blockers, and the dated buy/revise/stop decision. Keep a clear next action.

The immediate conversation to have

Ask a hospital insider: **“Think of the last major software purchase here. What happened from the first problem to the signed decision - and where did it nearly stop?”** Use the answer to correct our map rather than simply fill in a template.

What this changes for MediMind.

Our synthesis using the lecture and your existing parallel-system offer. These are recommendations, not reported results or lecturer-endorsed promises.

Keep the offer. Make the path to purchase explicit.

A free parallel system lowers the cost of trying. It does not remove the need for fit, a real problem, a sponsor, staff time, approvals or a buying decision. **No obligation to buy can coexist with a clear agreement about how to evaluate.**

Before the build

Confirm the hospital fits the chosen segment. Agree a bounded test, the problem being solved, who will use it, what good looks like and when the results will be reviewed. Confirm the technical/data route before starting the delivery clock.

During the evaluation

Help named staff complete real work in the parallel system. Check in, record friction, and compare against the baseline. Do not treat access granted, system delivered and staff actually testing as the same milestone.

After the evaluation

Review results with the people who can move the decision forward. Record a purchase path, a specific revision/retest, or a reason to stop. Avoid a free evaluation with no agreed end or decision.

What to track	What it tells us
First real tests at new hospitals	A near-term indicator that we are overcoming the current bottleneck: winning evaluations outside the anchor partner. It is not proof of a successful evaluation or sale.
Evaluations reaching a results review	Whether our test process gets to an evidence-based decision instead of drifting.
Pilot-to-paid conversion	Whether evaluated customers buy. Define the cohort, the evaluation milestone and the decision window; show the numerator/denominator and separate pending cases.
Reasons for delay, loss or revision	What to fix in qualification, product, onboarding, proof or the buying process.

Do not replace the focus metric automatically: first tests can remain useful for the present bottleneck; also check whether evaluations become customers. **Protect delivery capacity:** the lecture asks for reciprocal commitment when doing custom work (around 01:09-01:11). For our no-obligation offer, define bounded scope, evaluator time and a review date rather than inventing a purchase obligation. Track effort and reuse what repeats; intensive early support is not a permanent scaling model (00:21:44-00:23:26).

How this pack was made: on-device automatic transcription, review of timestamped text, and slide-text extraction with selected visual checks. The raw transcript is not fully verified word-for-word; names, room discussion and unclear phrases can be wrong. Refer to the original recording for exact quotations. Slides and recording are treated as private study material.